MINDFULLY YOURS · AI SERVICE

Knowledge Base Meeting Preparation Guide

What it is, how it works, what to explain, and exactly what to ask the clinician who built it

> **Meeting agenda (set by the client):** all knowledge-base-related topics — *AI logic*, *KB ingestion & routing*, and *conversations*.
>
> **Written for:** you, as the engineering lead, to carry the conversation with the clinical author of the knowledge base.
> **Prepared:** 5 October 2026 · **Source:** the project's 31 understanding documents and the current backend code.

0. How to use this guide

The client's message is short, but it names three areas. This guide gives you, for each area: (1) a plain-language understanding of how it works, (2) **what to explain** to the doctor, and (3) **what to ask** — with the reason each question matters and the answer you need to walk away with.

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Colour key used throughout

SAY THIS

Plain-language explanation you can give the doctor out loud.

ASK THIS

A question for the doctor. The answer changes something we build.

BE CAREFUL

A weak spot or honest limitation — better to raise it ourselves than have it discovered.

SAFETY-CRITICAL

Anything touching crisis handling. Needs the doctor's explicit sign-off.

THE ONE-SENTENCE GOAL OF THIS MEETING

Show the clinician that we have taken their knowledge base and turned it into working logic *without changing their clinical meaning*, and get their decisions on the specific gaps and judgement calls only a clinician can make.

Who you are talking to, and what they care about

The author is a clinician, not an engineer. They will care about: **(a)** is my clinical wording being preserved or paraphrased by the AI? **(b)** is the AI safe when someone is in danger? **(c)** does the AI stay inside what it is allowed to do (no diagnosing, no invented advice)? **(d)** where did you have to guess, and who decides? Avoid engineering vocabulary (embeddings, vectors, pgvector). Use "fingerprint of meaning" if you must, and lead with *clinical behaviour*.

1. The 5-minute story — what to say first

Open with this. It is the whole system in plain words, in the order a clinician will find natural.

SAY THIS — OPENING

"Everything you wrote — the signals, the red flags, the routing rules, the conversation samples, the question banks — is now inside a structured database. When a user types something, the system does three kinds of work, and each kind uses your material in a different way:

One — it learns *how to sound* from your sample conversations. That sets tone only. Nothing is looked up.
Two — it *searches* your material to work out what the person means (which signal, which emotion, which situation) **and to find a real exercise to hand back.**
Three — it follows your *rules* exactly — your routing table, your thresholds, your red-flag scripts — with no AI judgement involved.

The AI's only job is to phrase things warmly. It does not decide what is true, what is dangerous, or what to recommend. Those come from your content."

The three rules everything is built on

Rule	In plain words	Why a clinician will like it
1. Grounded, not invented	The AI may not make up mental-health advice from general knowledge. Every meaningful statement has to trace back to the client's approved material.	Clinical accountability: advice traces to your content.
2. Safety lives outside the AI	A separate, rule-based checker scans *every* message for danger. It is deliberately not "smart" so it behaves predictably, cannot be talked out of its job, and can override everything else.	Crisis responses are your exact scripts, not improvised.
3. Stay human on the outside	No forms, no quizzes. All severity assessment happens silently from ordinary conversation.	Matches the warm, unhurried style in your samples.

Two ideas that are easy to confuse — get these straight first

Tier (1 / 2 / 3)

How serious is it? Tier 1 = everyday stress, self-help is fine. Tier 2 = a professional would help. Tier 3 = urgent, a human is alerted now.

Layer (I – IV)

How deep into the conversation are we? Opening → exploring → reflecting → action planning. It has nothing to do with danger.

> **BE CAREFUL**
>
> The client's own documents also contain a *third* scale (Mild / Moderate / Severe / Crisis) in the "Pathway" document. We have **not** yet been told how it relates to Tier 1/2/3. This is one of the most important questions to ask (see Q-A1 in Section 6). Do not assume an answer in the meeting.

Where the AI sits in the product

The product has four layers: the app (what people see), the main backend (logins, bookings), **the AI service (what we are building)**, and secure storage in India (data-protection law). The AI service is deliberately a separate box so a bug elsewhere can never interfere with the safety checker. Only layer 3 is in our scope.

2. What the doctor gave us — the knowledge base inventory

This is the doctor's own work. Knowing the numbers lets you speak with confidence about *their* material. There were two drops: the six original clinical spreadsheets ("Phase I"), and the September 2026 working documents from the clinical team.

2.1 The six original clinical sheets (Phase I)

Sheet	Rows	What it contains	How the system uses it
A1 — Signal Library	285	Clinical "signs" (e.g. SIG-001 Anhedonia, SIG-002 passive suicidal ideation). Each has everyday phrasings, severity weight 1–3, category, linked disorder, red-flag yes/no.	Matching what the user says to a clinical signal. Severity weight feeds the running score.
A2 — Symptom Map	154	Physical and emotional symptoms (PHYS- and EM- codes) with how users describe them and which signals they map to.	Second matching source; EM- codes drive the Tier 1 (self-care) routing rules.
C11 — Routing Rules	66	If-then rules: which signal combinations route to Tier 1, 2 or 3, with priority 1/2/3.	Evaluated in priority order, no AI. Decides the tier.
7 — Red Flags	31	Danger phrases, clinical meaning, immediate action, a verbatim bot script, escalation target.	The safety interlock. Match → send the exact script, AI never called.
9 — Signal Sufficiency	185 raw / 195 merged	"How much evidence is enough" per disorder pattern, per tier: minimum signals, must-haves, when to keep probing, when to exit gracefully.	Tells the AI what to ask next and when it has enough to route.
17 — Fallback & Safety Net	99	What to do when a conversation goes off-script (4 sections + an 84-row disorder matrix).	Checked against the *shape* of the conversation (turn count, short answers, low confidence).

2.2 The September 2026 drop (46 working documents)

Content	Stored rows	What it is
Layer I–IV question banks	1,512	Questions/statements per conversational depth; Layer II–III are organised by emotion.
Scenarios taxonomy	2,809	Real-life situations and example statements. **270 of these are on hold** ("Fetishes" section) pending a governance decision.
Emotion phrasing	1,276	First-person "I feel…" phrasing from three different lists (19, 9 and 28 emotions — unreconciled).
Tier question banks	1,413	Open and close-ended follow-up questions per scenario and career topic.
Career lookup	500	Career → field → academic stream (deduplicated from a file that was one template copied 500 times).

“When AI is not sure” phrases	115	Clarifying questions, 16 categories.
Anxiety situations	35	Anxiety topic coverage checklist.
Parenting guardrail rules	18	Urgency rules for parenting concerns.
Self-help tools	5	The five DBT skills from the workbook. The only real “hand it back to the user” content so far.
Sample conversations (style)	~30 files	Teach tone only; not stored as searchable rows.

Totals: 21 source files ingested → 15 database tables → **8,513 rows**, of which 8,294 (97%) carry a meaning-fingerprint for matching.

> **SAY THIS**
> “We ingested all of it and kept every row tied back to the sheet and row you wrote. Nothing was rewritten. Where your files had gaps or inconsistencies, we did not guess — we parked those rows and made a list for you. That list is the second half of today.”

2.3 The three “buckets” — how every file is used

Each file is sorted by one question: *is it something a person might **say**, a real thing to **hand back**, or a **rule/number** the system checks?*

Bucket	What goes in	How it is used
1. Teach the AI how to talk	Sample conversations, the Clinical Markers table, the career-indecision template	Read once into the AI’s standing instructions. Shapes tone. Never searched during a chat.
2. Search, then match or recommend	Signal phrasings, symptom phrasings, red-flag trigger phrases, emotion phrases, scenario lists, layer question banks, DBT self-help tools, career lookup	Compared by *meaning* to what the user just said — either to understand what is going on (matching) or to find a real exercise to offer (recommending).
3. Become fixed rules	Routing rules, sufficiency thresholds, fallback triggers, the 15-step Pathway document	Plain if-then logic in code. Same input always gives the same answer. Auditable.

> **WHY NOT JUST SEARCH EVERYTHING?**
> Style does not vary by topic, so searching for “the right tone” each turn is pointless. And a routing decision must be *exactly* right, not “close enough” — so rules are checked directly, never fuzzily matched. Each technique is used only where it is the right tool.

3. Topic A — AI logic

This is the heart of the meeting. The doctor will want to know what the AI does on each message and, above all, where it is *not* allowed to decide.

3.1 What happens on every single message (11 steps)

1	Message arrives (typed, or voice converted to text)	transport
2	Exact cache: “have we seen this exact message before?”	fast lookup
3	Similar-message cache: “have we seen something very similar?”	meaning match
4	**Silent understanding** — read the message against your signals, symptoms and Clinical Markers. **Always runs.**	Bucket 2 + 1
5	**Safety check** — independently scan for danger phrases; can override everything below. **Always runs.**	Bucket 2 + 3 hybrid
6	Routing — do the accumulated signals match one of your rules? → Tier	Bucket 3 (rules)
7	Build the AI’s briefing: persona + relevant real material + what is known so far	Bucket 1 + 2 + 3
8	AI writes **one** warm reply — phrasing only	LLM
9	Guardrail: is the reply backed by real material?	check
10	Reply streamed back, word by word (text, voice or avatar)	transport
11	In the background: save a full audit record; update memory for next turn	async

THE RULE THAT CAN NEVER BE BROKEN

Steps 4 and 5 run on **every** message, even if a cached answer exists. Caching may skip the slow, expensive work — never the safety check. This is not a performance choice; it is the core safety guarantee.

3.2 The safety interlock — the most important thing to explain

SAY THIS

“When someone writes something like ‘I don’t feel like being here anymore’, the system compares its *meaning* to the trigger phrases you wrote for each red flag. If it matches closely enough, we do **not** ask the AI to write a reply. We fetch your exact bot script for that red flag and send it word for word. The AI is structurally prevented from being called at all.”

If the matched flag has **no script** (today: RF-028 to RF-031), the system does not improvise. It raises a hard alert to a human instead.

The thresholds — where clinical judgement and engineering meet

“Matching by meaning” returns a closeness score. We must pick how close is “close enough”. These values are **our engineering starting guesses from small manual tests — not clinically validated**:

Setting	Value	Purpose

General red-flag match	0.44	Deliberately permissive — a false alarm costs a moment of caution; a miss could cost a life.
Suicidal-ideation flags (RF-018, RF-019)	0.36	Even more sensitive, and checked among the 15 nearest candidates, because real disclosures scored only 0.37–0.41.
RF-025 (self-harm) stricter bar	0.50	Raised because ordinary anger (“I felt like hitting him”) falsely triggered it.
RF-028 (violence/unsafe home) soft zone	0.44–0.55	Below 0.55 it asks a gentle clarifying question instead of firing a crisis script (“I want to leave my partner” was falsely escalating).
Signal / symptom match	0.45	Matching what the user says to your signals.
Self-care recommendation match	0.30	Confidence needed before suggesting an exercise.

BE CAREFUL — RAISE THIS YOURSELF

We found two real problems while testing, both because of how matching by meaning behaves: **(1)** ordinary relationship or anger language falsely triggered red flags; **(2)** some genuine suicidal-ideation phrasings (“I want to kill myself”) initially *missed*. Both are patched, but a handful of test messages is not proof. We need a clinician-labelled test set (see Q-A3). One root cause is in the knowledge base itself: RF-025’s trigger phrases include “slamming my fist in the wall”, which pulls it toward anger-at-others language (see Q-B5).

ASK THIS

“We set our danger-detection to lean sensitive — an unnecessary ‘are you safe right now?’ is better than a missed disclosure. Do you agree with that trade-off for each flag, or are there flags where an unnecessary escalation is itself harmful (e.g. violence or abuse flags where a wrong escalation could put someone at risk)?”

3.3 The running tier score — how the AI “remembers” a conversation

Before this, “is it serious?” was decided fresh on every message. A caring listener doesn’t panic at one sentence, nor ignore a pattern. So each conversation now carries a hidden score from 0 to 100 that moves a little with each message.

Score	Tier	Meaning
0 – 39	Tier 1	Ordinary conversation; self-care chat continues.
40 – 100	Tier 2	Worth real attention: listen more carefully; suggest professional help only once the confidence gate clears.
—	Tier 3	**Never from the score.** Tier 3 means crisis and belongs only to the independent safety check.

What moves the score — only three things

1. **New, genuinely different signs of concern**, weighted by *your* severity ratings (mild +8, moderate +15, severe +25 points; unrated ≈ +4). Saying the same worry three ways counts once.
2. **Two questions, once the person has actually answered:** “how long has this been going on?” and “is it affecting daily life?” Each gives a real jump.

3. **Decay:** quiet, ordinary turns ease the score down. A rough patch that settles is not treated as permanently serious.

A cap limits how far any *single* message can move the score — added after one message matched nine signs at once and shot straight to 100.

Worked example (a real test)

Turn	What the user said	Score	Tier
1	“I’ve been feeling a bit stressed about work lately”	35	1
2	“It’s been going on for a few weeks now, I keep procrastinating” *(duration confirmed)*	60	2
3	“It’s affecting my performance at work, I missed a deadline” *(impact confirmed)*	95	2
4–5	Feeling unmotivated; stopped seeing friends	100	2

SAY THIS

“A single stressed sentence stays Tier 1. It took confirmation of duration and impact to climb. And even at 100 it stays Tier 2 — a serious pattern is not a crisis, and the system never confuses the two.”

The confidence gate — a high score is not permission to push booking

The bot only suggests booking a consultation when **all four** are true: (1) score is in Tier 2, (2) the person told us *how long*, (3) they told us it is *affecting daily life*, (4) *multiple distinct* concerns have appeared. Until then it keeps listening and gently gathering the missing piece. When the gate clears, the suggestion reads:

> “It might help to talk with a mental health professional who can support you through this in a safe, understanding space. Would you like help finding someone to speak with, or help with how to go about booking an appointment?”

BE CAREFUL

All scoring numbers (points per severity, the 40 cut-off, the gate conditions) are **our provisional guesses**. Also: the “how long / affecting life” check reads only the *current* message, not earlier turns, and cannot yet tell “I used to feel that way but I’m much better now” from an active complaint. Say so before they discover it.

ASK THIS

- “Is 40 a sensible line between ‘everyday’ and ‘worth a professional’? Would you put it higher or lower?”
- “Are duration and daily-life impact the right two conditions before suggesting a consultation? Is there a third you would always want (e.g. sleep, functioning, risk)?”
- “Would you approve the suggestion wording above, or rewrite it?”

3.4 Signals, evidence and “don’t trust one message”

One ambiguous message can match a signal weakly — a benign “I had a really productive day at work” scored higher against *Hypomania* than a genuine stomach-pain message scored against

Emotional Pain. So matches **accumulate across turns** and nothing raw ever reaches the AI as fact. The AI is shown only two things: a confirmed Tier 1 self-care recommendation, or **your own “keep probing” guidance** for the closest pattern — an instruction for *what to ask next*, never a diagnosis or the pattern’s clinical name.

SAY THIS

“The AI never tells a user they might have a condition. It only asks the next question your Signal Sufficiency sheet says to ask.”

ASK THIS

“Your sufficiency sheet’s ‘when to keep probing’ text is what steers the AI’s next question. Is that text written as an instruction to the AI, or as a note to a human clinician? Does it read safely if the AI paraphrases it?”

4. Topic B — KB ingestion & routing

"Ingestion" is how raw spreadsheets and Word files became a searchable database. "Routing" is how signals become a tier. The doctor will care about two things: *was my content preserved?* and *did your reading of my rules match my intent?*

4.1 Ingestion in plain words

1	**Open the raw file** and split it along its own structure (rows, headings, sections). The client's structure *is* the chunk boundary — one row or one section is one complete idea. AI is not used to decide splits.	structure-aware parsing
2	**Clean & validate** — remove template instruction rows, fix encoding, fill in labels written once per group, normalise ID formats (`SIG: 001`, `SIG-168`, `SIG002-003` are all the same style of reference).	cleaning
3	**Tag** each piece with category and language.	metadata
4	**Fingerprint** only the real human phrasing (e.g. signal phrasings, red-flag trigger phrases) so it can be matched by meaning.	embedding
5	**Store** in one database; the fingerprint sits in the same row as the text.	PostgreSQL
6	**Version & roll out safely** — every run is a labelled version; it goes live only after validation, and can be rolled back.	versioning

SAY THIS

"Nothing is rewritten. We kept your wording exactly. The only addition is a numeric fingerprint stored beside your text. Every row traces back to the file and row you authored."

4.2 How routing works

Routing turns signals into a tier. It is **pure rules, no AI**. Example (RULE-001): *"For Major Depressive Disorder (SIG002-003), no other signal required" → Tier 3, Priority 1.*

- Rules are evaluated in **Priority** order (1, then 2, then 3) — **not** in Rule-ID order. They genuinely differ in your data (e.g. RULE-004 is priority 1, so fires before RULE-002/003).
- The first rule whose condition matches the signals accumulated *so far this conversation* wins.
- All seven priority-3 rules (Life Transitions, Interpersonal skills, Identity, Existential crisis, Career/financial, Motivation, Self-improvement) route to Tier 1 and fire last.
- The Fallback sheet is checked against the *shape* of the conversation (e.g. 15+ turns, short answers, low confidence) — no text inspection.

ASK THIS

"Your sheet says 'Rule 1 is applied first', but the Priority column is what matters, and it doesn't always follow rule number. Is Priority the intended order of evaluation?" (We believe yes. Confirm — this decides which tier wins in a conflict.)

4.3 What we found in the data — the gaps only the doctor can close

#	Finding	Effect today	Need from doctor

1	**4 red flags with no bot script** — RF-028 violence/unsafe environment, RF-029 history of violence, RF-030 suspected rape, RF-031 sexual molestation	Match → hard alert to a human; AI never improvises. Cannot ship as-is.	Verbatim scripts
2	**41 of 66 routing rules** use shorthand (“SIG 001–017 minus SIG002–003”, “EM45 to 54”) that our reader can’t fully expand	Skipped rather than guessed. Only ~25/285 signals auto-link to a rule.	Spell out each signal, or confirm shorthand meaning
3	**Rule-025** (Factitious Disorder) has a trigger but no route, priority or reason	Switched off.	Complete the row
4	**3 sufficiency rows** say “check----------” instead of a tier (Recurrent Mood Episode; major depressive episodes; harm to self/others with manic episode)	Excluded from matching.	The correct tier
5	**RF-020** (Psychotic Feature) has no escalation target	Action is Tier 3 but destination is blank.	Escalation target
6	**Sufficiency sheet: 185 vs 195 rows** in two exports	We built on the larger, newer one.	Which is authoritative
7	**A named individual** is written into 3 fallback escalation paths where all other rows use a role	Privacy and staffing risk if that person leaves.	Replace with a role
8	**Typos in word-for-word scripts** (e.g. “Ill be right here”, “Would to be comfortable”)	Would ship as written.	Corrected versions
9	**“Guardrail Questions”** is listed in the file directory but never received as its own file	May be part of Fallback sheet.	Confirm
10	**Incomplete conversation files** (Trauma, Depression, Bipolar & Psychosis, Life Transitions, Parenting) and **3 password-locked files**	Used only for what exists.	Complete, or confirm intentional; unlock password

BE CAREFUL — TONE

These are not criticisms. Real clinical spreadsheets have exactly these gaps. Frame each as “here is what we parked rather than guess”. That shows the doctor you protected their intent.

4.4 The sensitive content — handle with care

SAFETY-CRITICAL / GOVERNANCE

The scenarios document contains a “Fetishes” section covering all eight DSM-5-TR paraphilic disorders, including content framed as non-offending, help-seeking material. We loaded it (270 rows) but **flagged it “pending governance review” and excluded it from any live use**. It needs an explicit clinical *and* legal decision first.

ASK THIS

“For the paraphilic-disorder section: should the AI always hand off to a human clinician? Never engage? Engage with a fixed script only? And who on the legal side should confirm?”

4.5 How we know ingestion worked

Each file was inspected against its real structure before a parser was written, then verified by row-count against expectation. Real issues this surfaced: hidden template rows that looked like data; a Word file built entirely from drawing text-boxes; duplicated text from a rendering quirk; and a column whose title used a typographic dash that made a parser silently miss every safety script. Two of these (the dash bugs) would have meant **27 of 31 red flags replying with generic text instead of your verbatim script** — caught and fixed before any real use.

5. Topic C — Conversations

This is where the doctor's expertise is most visible. They will want to know whether the AI sounds like their samples and whether it respects the structure they designed.

5.1 The persona — what the samples teach

From the 18 sample dialogues across 9 scenarios (job loss, career confusion, family pressure, exam failure, burnout, workplace friction, etc.):

- **Warm, brief acknowledgement** then an **open invitation** to share ("Hey, that sounds difficult… Would you like to share what has been happening?").
- **Funnel of questions:** broad → clarify timeline → name emotions → intensity/duration → impact on sleep/work/relationships → self-blame → a small concrete step → open-door close ("I'm here if you need me").
- **Reflect and validate**; ask where the emotion is felt in the body.
- **Small, time-boxed experiments** with a scheduled follow-up ("for the next 3 days and get back to me").
- **Knows its scope:** for "which stream / which job?" it declines concrete advice and redirects to emotional support or a career counsellor.

BE CAREFUL

The original sample file contains **no crisis conversation**. Crisis handling comes only from your red-flag verbatim scripts. The September drop does add some real crisis-moment examples, but they are not yet the model for crisis replies.

ASK THIS

• "Are the sample conversations a **script** (to follow closely) or a **style guide** (to capture tone)? Our design assumes tone only."
• "Which 5–15 samples best represent the voice you want? We will use these as the fixed examples shown to the AI every time."
• "Which sample would you *not* want the AI to imitate?"

5.2 Layers I – IV — conversational depth

Layer	Role in a conversation	Content we hold
I	Opening — rapport, what brings you here	156 questions/statements + Phase II Layer I & IV (184)
II	Exploring — emotion-specific questions	576 (28 emotions)
II → III	Transition from exploring to reflecting	81
III	Reflecting — emotion-specific reframes	515 (same 28 emotions)
IV	Action planning / close	within Phase II Layer I & IV (184)

Design intent: the system picks the closest-fitting phrase from your bank for "where are we now" and gives it to the AI as **phrasing material** — the AI never invents the question from scratch.

HONEST STATUS

These layer question banks are ingested and searchable, but **not yet wired into live conversation**. They are waiting on the emotion and Tier/Layer decisions below. Say this plainly.

ASK THIS

• “What decides when a conversation moves from Layer I to II to III to IV — number of turns, user readiness, or something the AI should detect?”
• “Can the conversation go backwards (e.g. back to Layer I) if the person says something new?”
• “Does Layer interact with Tier at all — e.g. does a Tier 2 conversation skip any layer?”

5.3 Emotions — three lists that disagree

The documents contain three emotion lists: **19** items (Phase II expression file), **9** items (List of Emotions), and **28** items (used identically in Layer II and Layer III). We store all three tagged separately and deliberately left the “canonical group” empty rather than guess.

ASK THIS

• “Is the 28-item list your intended final list?”
• “Which names mean the same thing? (e.g. ‘Tensed’ vs ‘Anxiety’, ‘Concerned’ vs ‘Worry’.) Which do we keep?”
• “Can you send one confirmed list?”

5.4 Scenarios — two lists that disagree

A 30+ item list (“Relationship Issues”, “Career Issues”…) and a ~22-item list in the Pathway document, plus a section titled “Scenarios That Were Left” (10 further categories).

ASK THIS

“Should these be merged into one list? If they conflict, which wins? Is ‘Scenarios That Were Left’ meant to merge in or stay separate?”

5.5 The Pathway document — the AI’s decision flow

The 15-step “Pathway For AI In Different Situations” is the closest thing to pseudocode the client produced. It is not stored as data; it is the **blueprint of the conversation logic**: identify concern → categorise → safety check → assess emotion and severity → choose intervention → close.

ASK THIS

• “The Pathway uses Mild/Moderate/Severe/Crisis. Is that replacing Tier 1/2/3, alongside it, or an earlier draft?”
• “Does ‘Crisis’ equal Tier 3, or are they different outcomes? (Two of your three architecture documents say 3 states; one says 4 — Tier 1/2, Tier 3 practitioner booking, and Crisis.)”
• “Walk us through one real example from start to finish so we can confirm our implementation matches your intent.”

5.6 Self-help tools — what the AI can hand back

Currently five DBT skills from the workbook are searchable (e.g. “PLEASE” for physical self-care). A brain-dump worksheet is to be shown as a downloadable asset rather than searched. At Tier 1 the AI searches for the best-fitting tool; the AI phrases the offer, it does not choose or invent the exercise. If nothing matches closely enough, it falls back to a safe generic reply rather than stretching a weak match.

GAP

There is no **mapping** from “this concern/emotion” to “this exercise”. Only five tools exist. This is a content gap, not an engineering one.

ASK THIS

• “For each common Tier 1 concern (low motivation, anxiety, sleep, loneliness, burnout), which exercise would you want offered first?”
• “Is there more self-help content coming (Phase III psycho-education, Phase IV tools) and when?”
• “Which tools should *never* be offered without a clinician (e.g. exposure-type exercises)?”

5.7 Different contributors, different conventions

Some scripts tag Tier 1/2/3, others use none or a bespoke label (“Chronic risk”). Before using these to calibrate routing the team needs one standard.

ASK THIS

“Can all authors adopt the same tiering for every script from now on? Who owns that standard?”

6. Master question list for the doctor (prioritised)

P1 = blocks a safety-critical or structural feature; get an answer in this meeting. **P2** = needed before the affected feature ships. **P3** = housekeeping; email afterward.

A. AI logic

ID	Pri	Question	Why it matters	Answer we need
A1	P1	How do Tier (1/2/3), Layer (I–IV) and Severity (Mild/Moderate/Severe/Crisis) relate? Is Crisis = Tier 3? 3 outcomes or 4?	Sets the number of routing outcomes and the database design.	A one-line definition of each, and the mapping.
A2	P1	For each red flag, is it better to escalate unnecessarily or risk missing? Any flags where wrong escalation is harmful?	Tunes sensitivity per flag.	A sensitivity stance per flag group.
A3	P1	Will clinicians label a test set — a few hundred real-sounding messages with the tier or flag they would assign, including decoys that sound alarming but are not?	Current thresholds come from ~9–15 manual tests. This is the only way to validate them.	Yes/no, who, and by when.
A4	P2	Is 40/100 the right Tier 1/Tier 2 line? Are duration and daily-life impact the right two gating conditions? Add a third?	Provisional scoring numbers.	Clinician's adjustment, or approval.
A5	P2	Approve or rewrite the "consider a professional" suggestion wording.	This is the highest-stakes non-crisis sentence the AI says.	Approved wording.
A6	P2	How should "I used to feel this way but I'm better now" be treated? Should earlier-confirmed duration carry over?	Known limitation: only the current message is read.	Clinical rule of thumb.
A7	P2	Is the "keep probing" text written for the AI or a human? Safe if paraphrased?	It steers the AI's next question.	Confirm, or rewrite for AI use.
A8	P2	Should anything be off-limits for the AI regardless of tier (diagnosis, medication, specific career choices)? Is the list in the samples complete?	Scope boundaries.	An explicit "never do" list.

B. KB ingestion & routing

ID	Pri	Question	Why it matters	Answer we need
B1	P1	RF-028 to RF-031 have no bot script (violence, history of violence, rape, molestation). Can you write the verbatim script for each?	Highest-severity categories currently fall back to a human alert only.	4 verbatim scripts.
B2	P1	RF-020 (Psychotic Feature) has no escalation target. Who is it?	Tier 3 action with no destination.	A named *role*.

B3	P1	What is each escalation target in practice — who is on-shift, how fast, through what channel?	“Psychiatrist + crisis review” is a label; we need the real handoff.	Roles, response expectations.
B4	P2	41 routing rules use ranges/exclusions. Rewrite with every signal listed, or should we build a smarter reader for the shorthand? (e.g. what exactly does “SIG 001–017 minus SIG002–003, at least 5 for 2 weeks” include?)	Today these rules are skipped, not guessed.	A choice, plus clarification of 3–5 sample rules.
B5	P2	RF-025’s trigger phrases include “slamming my fist in the wall”. Can we reword it and add more self-harm-specific examples?	Currently causes anger-at-others false alarms.	Revised trigger phrases.
B6	P2	Rule-025 is missing route, priority, reason. Please complete.	Rule is switched off.	Completed row.
B7	P2	Three sufficiency rows say “check----------”. Correct tiers for those three patterns?	Excluded from matching.	Tier 1, 2 or 3 for each.
B8	P2	Sufficiency sheet: 185 or 195 rows — which is correct?	Version control.	The authoritative file.
B9	P2	Is priority (1→2→3) the intended evaluation order rather than rule number?	Decides conflicts.	Yes/no.
B10	P2	Paraphilic-disorder section: what should the AI do, and who in legal confirms?	270 rows held out of live use.	A handling rule.
B11	P3	Replace named individuals in 3 fallback rows with roles.	Privacy / staffing risk.	Role names.
B12	P3	Send corrected versions of scripts with typos.	Scripts go out verbatim.	Corrected text.
B13	P3	Is “Guardrail Questions” its own file, or inside the Fallback sheet? Is the 4-section + 84-row Fallback structure complete?	Possible missing file.	Confirmation.
B14	P3	Password for Anam’s 3 locked files.	Cannot be reviewed.	Password or re-send.
B15	P3	Which versions of the incomplete conversation files (Trauma, Depression, Bipolar & Psychosis, Life Transitions, Parenting) are final? Will they be completed?	Some topics listed but never written (e.g. paranoia).	Completion plan.

C. Conversations

ID	Pri	Question	Why it matters	Answer we need
C1	P2	Sample conversations: script or style guide? Which 5–15 are the best voice examples?	Shapes every AI reply.	Stance + shortlist.

C2	P2	Which emotion list is final (19, 9 or 28)? Which names are synonyms?	Cannot wire emotion matching until settled.	One confirmed list.
C3	P2	Merge the 30+ and ~22 scenario lists? What about “Scenarios That Were Left”?	Cannot route a conversation to a topic cleanly.	One list.
C4	P2	What triggers movement between Layers I → II → III → IV? Can it go backwards?	Layer question banks are ready but not wired.	The rule for moving layers.
C5	P2	Which self-help exercise for which concern?	No mapping exists; only 5 tools.	A concern → exercise table.
C6	P2	When more Phase III/IV content is coming, and what format?	Tier 1 helpfulness depends on it.	A date and contents list.
C7	P3	Standardise tier labels across all authors’ scripts.	Needed before using scripts to calibrate.	Agreement on one standard.
C8	P3	Is Hindi in a later phase, and will the KB be translated by your team?	Phase 1 is English only by design.	Timing and ownership.

7. Honest status: built vs. not built

Be exact. Doctors prefer “this part is real, this part is not” over optimism. This table reflects the latest code and documents; earlier overview documents describe some items as unbuilt that have since been built.

Component	Status	In plain words
Ingestion of all received files	Done	21 files → 15 tables → 8,513 rows; verified.
Database design	Done	All tables built; versioned loading.
Safety interlock (red flags)	Working	Verbatim scripts for 27 of 31 flags; hard alert for the other 4. Thresholds provisional; false-positive/negative fixes in place.
Signal and symptom matching	Working	Matches accumulate over the conversation; noisy at single-message level by design.
Running tier score + confidence gate	Built, provisional	Works as designed on tests; every number awaits clinical calibration. Tier 3 deliberately unreachable by score.
Routing rules	Partial	Rules with plain references work; 41 of 66 use shorthand that is skipped; Rule-025 off.
Real AI reply + conversation pipeline	Working	Streaming, grounded in retrieved content or your probing guidance; ~1.7 s on a tested turn (target 1.5–2.5 s).
Voice (speech in/out, live room)	Working	Live-tested end to end; latency per stage not yet instrumented.
Output guardrail (is the reply grounded?)	Basic	A simple check exists; a thorough one is still to do.
Self-care recommendations (Tier 1)	Mechanism built	Code works; only 5 DBT skills exist and no concern → exercise mapping.
Layer I–IV question flow	Not wired	Content ingested; waiting on Tier/Layer/emotion decisions.
Style examples in the AI’s briefing	Not wired	Slot exists but empty until the 5–15 examples are chosen.
Clinical validation test set	Not started	Needs clinician-labelled examples; the key to trusting every threshold.

FRAME IT THIS WAY

“Everything about *organising and storing* your knowledge is done. The safety path and the basic conversation work for real. What is left is mostly decisions only you can make, content only you can write, and your clinical review of every number we picked.”

8. Likely questions from the doctor, with answers

“Does the AI rewrite my scripts?”

No. For red flags and fallback scenarios, your *bot script* is sent word for word and the AI is not called. Elsewhere, the AI phrases a reply using your content as grounding. The reason we asked about script typos is that we will not silently edit your clinical wording.

“Can the AI diagnose?”

No. It is never shown a diagnosis name. It is shown only your “what to ask next” guidance. Signal matches are internal and never stated to the user.

“What if the user says something dangerous and the system misses it?”

It can happen: matching by meaning is not perfect. That is why thresholds lean sensitive for suicidal-ideation flags, and why we want a clinician-labelled test set. We found and fixed real misses during testing (“I want to kill myself” originally fell just under the threshold). We would rather over-check than miss. We are not claiming it is validated yet.

“What if it falsely flags an ordinary message?”

It happened with relationship and anger language (“I want to leave my partner”). The violence/unsafe-home flag now has a soft zone that asks a clarifying question instead of firing a crisis script. Suicide flags deliberately have no soft zone.

“Why is Tier 3 not part of the score?”

Our first version let the score reach “Tier 3” at 70+. A real work-stress conversation hit it, which was wrong: a serious pattern is not a crisis. Tier 3 now means exactly one thing and is handled only by the independent safety check.

“Is my data safe?”

The AI service is designed for Indian data-protection requirements, storage in India, an audit log of every turn, and vendors required not to train on or retain the data. One caveat: the embedding vendor was changed (the originally named vendor has no embeddings product), which needs the client’s written sign-off before real user data is embedded.

“How long until it is ready?”

Do not promise a date. Say: the engineering path is clear; the critical path is clinician decisions (A1, A3, B1–B3), the missing scripts, and the validation set.

“Who decides when a number is wrong?”

Proposed: we run a batch of sample conversations, you label the tier you would assign, and we adjust the numbers wherever we disagree — repeated until the AI’s answers reliably match your judgement.

9. Suggested agenda and “leave with” checklist

Time	Segment	What you do
0:00 – 0:10	Frame & the 5-minute story	Section 1. Confirm the three topics. Introduce Tier vs Layer.
0:10 – 0:25	Your knowledge base, as we hold it	Section 2. Show the numbers. Stress: nothing rewritten.
0:25 – 0:55	Topic A — AI logic	Safety interlock first, then the running score, then the gate. Ask A1–A5.
0:55 – 1:20	Topic B — ingestion & routing	How data was handled; the 10 gaps. Ask B1–B10.
1:20 – 1:45	Topic C — conversations	Persona, Layers, emotions, scenarios, self-help. Ask C1–C6.
1:45 – 2:00	Decisions & next steps	Run the checklist below. Agree owners and dates.

If time is short, protect: A1, A3, B1–B3, B10. These are the safety-critical ones.

Leave the meeting with these (tick each)

☐	Decision or deliverable	Owner	Due
☐	Definition of Tier / Layer / Severity and the mapping (A1)		
☐	Verbatim scripts for RF-028 to RF-031 (B1)		
☐	Escalation target for RF-020, and real-world escalation roles (B2, B3)		
☐	Agreement to a clinician-labelled validation set: who, how many, when (A3)		
☐	Approval or edits to the 40-point line, gate conditions and the “see a professional” wording (A4, A5)		
☐	Rewrite or clarify of the 41 shorthand routing rules (B4); Rule-025 (B6); 3 “check” rows (B7)		
☐	Clinical + legal ruling for paraphilic section (B10)		
☐	One emotion list, one scenario list (C2, C3)		
☐	Rule for moving between Layers (C4)		
☐	Concern → self-help exercise mapping, and timeline for more content (C5, C6)		
☐	Style-sample shortlist and script-vs-style answer (C1)		
☐	Password for 3 locked files; corrected script typos; roles instead of names (B14, B12, B11)		

Do and don’t in the room

Do

- Lead with clinical behaviour, not technology.
- Show a real example, then ask the question it raises.
- Admit provisional numbers before being asked.
- Write every decision down with an owner and date.
- Ask for a worked example when a rule is unclear.

Don’t

- Promise the system is “validated” or “safe” — it is not yet validated.
- Guess the meaning of shorthand rules in the room.
- Offer to “fix” clinical wording ourselves.
- Discuss vendors, payments or avatar choice — that is a separate meeting.
- Commit to dates that depend on clinician input.

10. Glossary — plain English

Term	Meaning
Tier 1 / 2 / 3	How serious it is. 1 = everyday stress; 2 = would benefit from a professional; 3 = urgent, a human is alerted now.
Layer I–IV	How deep the conversation has gone. Unrelated to danger.
Signal (SIG-xxx)	A clinical sign from the Signal Library, with everyday phrasings and a severity weight 1–3.
Symptom (PHYS-/EM-)	A physical or emotional symptom from the Symptom Map.
Red flag (RF-xxx)	A danger phrase. If matched, the exact pre-written safety script is sent; the AI is not called.
Routing rule	An if-then rule from the C11 sheet that turns signals into a tier.
Signal sufficiency	How much evidence is enough before routing, plus what to keep asking and when to exit.
Fallback	What to do when a conversation goes off-script (repetitive, short answers, low confidence).
Interlock	The independent safety checker that runs on every message and can override everything.
Verbatim / exact-fetch	Sent word for word, never rephrased by the AI.
Embedding (“fingerprint”)	A numeric representation of a phrase’s meaning, so different wordings of the same idea can be matched.
RAG	Search approved material first, then have the AI phrase an answer from it rather than invent one.
Threshold	How close a match must be to be trusted. Currently provisional.
Ingestion	The one-time conversion of raw client files into clean, searchable database rows.
Confidence gate	The four conditions that must all be true before the bot suggests booking a consultation.
Pending governance review	Stored but intentionally not used live until a clinical and legal decision is made.
Bucket 1 / 2 / 3	Our sorting of all content: teach tone / search then match or recommend / fixed rules.

Sources: the project’s understanding documents (01–31), principally 05, 15, 19, 20, 21, 22, 24, 26, 27, 29, 31, and the current backend code. Where an overview document disagreed with a later detailed document or the code, the later source was used. Threshold values and scoring numbers are provisional and may change as testing continues.